

cyclogest[®] 200mg and 400mg

Pessaries



1 Trade Name of the Medicinal Product
CYCLOGEST

2 Qualitative and Quantitative Composition
Each pessary contains either 200mg or 400mg of the active ingredient progesterone.

3 Pharmaceutical Form
Off-white pessaries.

4.1 Therapeutic Indications
Cyclogest is indicated for the
1) Treatment of premenstrual syndrome, including premenstrual tension and depression.
2) Luteal phase support as part of an Assisted Reproductive Technology (ART) treatment for women. **(Only for Cyclogest 400mg pessary)**

4.2 Posology and Method of Administration
For the treatment of premenstrual syndrome:
200mg daily to 400mg twice a day, by vaginal or rectal insertion. For premenstrual syndrome commence treatment on day 14 of menstrual cycle and continue treatment until onset of menstruation. If symptoms are present at ovulation commence treatment on day 12.

For luteal phase support as part of an ART treatment:
400 mg administered vaginally twice a day starting at oocyte retrieval. The administration of Cyclogest should be continued for 38 days, if pregnancy has been confirmed.

Use in special populations:
There is no experience with use of Cyclogest in patients with impaired liver or renal function.

Paediatric population:
There is no relevant use of Cyclogest in the paediatric population.

Elderly:
No clinical data have been collected in patients over age 65.

Method of Administration:
For rectal or vaginal insertion.

4.3 Contraindications

- Hypersensitivity to the active substance or to any of the excipients listed in section 6.1.
- Undiagnosed vaginal bleeding.
- Known or suspected progesterone sensitive malignant tumours.
- Porphyria.
- Severe hepatic dysfunction or disease
- Known missed abortion or ectopic pregnancy.
- Active arterial or venous thromboembolism or severe thrombophlebitis, or a history of these events.

4.4 Special Warnings and Precautions for Use
Cyclogest is not indicated in threatened miscarriage. Treatment should be discontinued in the event of a missed miscarriage.

Cyclogest should be discontinued if any of the following conditions are suspected:
myocardial infarction, cerebrovascular disorders, arterial or venous thromboembolism (venous thromboembolism or pulmonary embolism), thrombophlebitis or retinal thrombosis.

Although risk of thromboembolism has been associated with estrogens, a link with progestins remains questionable. Therefore, in women with generally recognised risk factors for thromboembolic events, such as personal or family history, treatment with Cyclogest may further increase the risk. In these women, the benefits of Cyclogest administration need to be weighed against the risks. It should be noted however, that pregnancy itself carries an increased risk of thrombo-embolic events.

Patients with a history of depression need to be closely observed. Consider discontinuation if symptoms worsen.

Because progesterone may cause some degree of fluid retention, conditions that might be influenced by this factor (e.g. epilepsy, migraine, asthma, cardiac or renal dysfunction) require careful observation.

A decrease in glucose tolerance has been observed in a small number of patients on estrogen-progestin combination drugs. The mechanism of this decrease is not known. For this reason, diabetic patients should be carefully observed while receiving progestin therapy.

Progesterone is metabolised in the liver and should be used with caution in patients with hepatic dysfunction.

Cyclogest contains the hormone progesterone which is present in significant concentrations in women during the second half of the menstrual cycle and during pregnancy. This should be borne in mind when treating patients with conditions that may be hormone-sensitive.

Treatment of premenstrual syndrome and depression:
Use rectally if barrier methods of contraception are used.
Use rectally if patients suffer from vaginal infection (especially moniliasis) or recurrent cystitis or have recently given birth.
Use vaginally if patients suffer from colitis or faecal incontinence.

4.5 Interactions with other Medicaments and other forms of Interaction
Drugs known to induce the hepatic cytochrome-P450-3A4 system (e.g. rifampicin, carbamazepine or phenytoin) may increase the elimination rate and thereby decrease the bioavailability of progesterone.
The effect of concomitant vaginal products on the exposure of progesterone from Cyclogest has not been assessed and is therefore not recommended.

4.6 Pregnancy and Lactation
Pregnancy
Cyclogest should not be used during pregnancy except as indicated during the first trimester of pregnancy for use as part of an assisted reproduction (ART) treatment (see section 4.1 for full details). There is limited and inconclusive data on the risk of congenital anomalies, including genital abnormalities in male or female infants, following intrauterine exposure during pregnancy. The rates of congenital anomalies, spontaneous abortion and ectopic pregnancies observed during the clinical trial were comparable with the event rate described in the general population although the total exposure is too low to allow conclusions to be drawn.

Lactation
Progesterone is excreted in human milk and Cyclogest should not be used during breast-feeding.

4.7 Effects on Ability to Drive and Use Machines
None known.



Riverside Drive | Cleckheaton
West Yorkshire | BD19 4DH | United Kingdom
Tel: +44 (0) 127 484 8000
www.sgkinc.com

Job No.
105095774_402256014

Item (JDE) Number
50998825

Dimensions
148x420 (Pre-Folded)

Pharma Code
1807

Wave ID Number
20108517

Version No.
01

Date
26 FEB 2021

Operator
BS

Printing Colours



Process Black

Technical Colours



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4.8 Undesirable Effects

Adverse reactions in patients undergoing luteal support as a part of ART treatment is presented in the table below:

SYSTEM ORGAN CLASS	Common ≥1/100 to <1/10	Uncommon ≥1/1000 to <1/100
Neoplasms benign, malignant and unspecified (incl cysts and polyps)		Rectal neoplasm
Psychiatric disorders		Mood altered
Nervous system disorder	Somnolence	Headache, dizziness, dysgeusia
Vascular disorders	Hot flush	Haemorrhage

Gastrointestinal disorders	Abdominal distension, abdominal pain, constipation.	Diarrhoea, vomiting, flatulence, gastric dilatation.
Skin and subcutaneous tissue disorders		Rash, pruritus, night sweats.
Musculoskeletal and connective tissue disorders		Arthralgia
Renal and urinary disorders		Pollakiuria, incontinence.
Reproductive system and breast disorders	Breast pain	Vaginal haemorrhage, pelvic pain, metrorrhagia, ovarian enlargement, vulvovaginal pruritus.
General disorders and administration site conditions	Fatigue	Feeling cold, feeling of body temperature change, application site pruritus, discomfort.
Investigations		Weight increased

When treating premenstrual syndrome, menstruation may occur earlier than expected, or, more rarely, menstruation may be delayed.

Soreness, diarrhoea and flatulence may occur with rectal administration.

As with other vaginal and rectal preparations, some leakage of the pessary base may occur.

4.9 Overdose

There is a wide margin of safety with Cyclogest pessaries, but overdosage may produce euphoria or dysmenorrhoea. Symptomatic treatment recommended.

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Sex hormones and modulators of the genital system; Progestogens; Pregnen-(4) derivatives. ATC code: G03DA04.

Progesterone is a naturally occurring steroid that is secreted by the ovary, placenta, and adrenal gland. In the presence of adequate estrogen, progesterone transforms a proliferative endometrium into a secretory endometrium. Progesterone is necessary to increase endometrial receptivity for implantation of an embryo. Once an embryo is implanted, progesterone acts to maintain the pregnancy.

Clinical efficacy and safety

In a Phase III clinical trial in pre-menopausal women subjected to ART and IVF the pregnancy rates after vaginally applied Cyclogest pessary (400 mg twice daily) was found to be 38.3% (FAS) and 38.1% (PP) after 38 days of luteal phase support. The clinical pregnancy rate was 34.5% after 70 days of luteal phase support.

5.2 Pharmacokinetic properties

Absorption

Vaginal administration of Cyclogest 400 mg every 12 h in healthy women has been shown effective in rapidly achieving and maintaining serum progesterone concentrations at physiological levels appropriate to the midluteal phase of the ovarian cycle and early pregnancy.

The mean Cmax after 10 days of multiple dosing was 18.4 [ng/mL] and Ctrough was 10.5 [ng/mL].

Distribution

Progesterone is approximately 96 % to 99 % bound to serum proteins, primarily to serum albumin and corticosteroid binding globulin.

Biotransformation

Progesterone is metabolized primarily by the liver largely to pregnanediols and pregnanolones. Pregnanediols and pregnanolones are conjugated in the liver to glucuronide and sulfate metabolites. Progesterone metabolites that are excreted in the bile may be deconjugated and may be further metabolized in the gut via reduction, dehydroxylation, and epimerization.

Elimination

Progesterone undergoes renal and biliary elimination.

5.3 Preclinical safety data

Not applicable.

6.1 List of excipients

Also contains: vegetable fat.

6.2 Incompatibilities

None known.

6.3 Shelf-life

Forty eight months

6.4 Special precautions for storage

Store below 30°C in a dry place.

6.5 Nature and contents of container

Thermoplastic film strips
Pack size: 15

6.6 Instructions for use/handling

Not applicable.

7 Manufacturer

ACCORD-UK LIMITED
Whiddon Valley, Barnstaple,
EX32 8NS, UK.

8 Date of Last Revision

February 2021

LDCollins

L.D. Collins & Co. Ltd., 1st Floor, Gallery Court,
28 Arcadia Avenue, London, N3 2FG, UK

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